entered Indian discourse because of a convergence of genuine scientific advances—especially GLP-1 receptor agonist drugs and continuous glucose monitoring—alongside commercial and social media promotion and rising health anxiety," says Dr Anoop Misra, chairman, Fortis C-DOC Centre of Excellence for Diabetes, Metabolic Diseases and Endocrinology.

Pharmaceutical giant Eli Lilly's Mounjaro clocked Rs 100 crore in October 2025 alone. Novo Nordisk followed with the launch of Ozempic—prescribed primarily for diabetes—in December 2025 and cut prices of its obesity drug Wegovy. Rybelsus, too, has seen explosive growth, with sales jumping from Rs 47 crore to Rs 363 crore in a year of its launch three years ago.

New drugs are poised to enter the market too. One eagerly awaited diabetes/ weight loss drug expected to enter India is Orforglipron, from Eli Lilly. Unlike the current Semaglutide/ GLP-1 drugs that are injectable, it is fully oral and, if approved, could corner a huge market in India. Drugmakers are also advancing triple-hormone agonists that simultaneously act on GLP-1, GIP and glucagon (another hormone that maintains energy balance by raising glucose levels), delivering faster fat loss and stronger improvements in cardiometabolic markers. Others include triple-agonists like Retatrutide, which has shown average weight loss exceeding 23 per cent in trials.

PRE-PILL PROTOCOL

The checklist doctors recommend

- **BLOOD SUGAR TESTS (HBA1C/ FASTING GLUCOSE):** Confirms diabetes or pre-diabetes status
- **EYE EXAMINATION** Rapid glucose improvement can worsen existing retinal disease
- **KIDNEY FUNCTION TESTS (CREATININE, EGFR):** Vomiting or dehydration on metabolic drugs can strain kidneys
- **THYROID HISTORY:** To check if patients/ close family members have thyroid cancer or MEN2
- **ABDOMINAL ULTRASOUND:** Screens for gallstones or fatty liver, which can flare up during rapid weight loss

"Indians reach metabolic risk at younger ages and lower BMIs... But that doesn't mean every overweight person needs a GLP-1 drug. Clinical thresholds and cardiometabolic risk must guide treatment"

DR ANOOP MISRA, *Chairman, Fortis C-DOC Centre of Excellence for Diabetes, Metabolic Diseases and Endocrinology*



THE PILL ISN'T ENOUGH

The looming health disaster arising from these 'weight-loss drugs' are forcing a rethink among doctors. Dr Prasad Biware, head of the department and Internal Medicine consultant at Manipal Hospital, Pune, explains: "We now understand metabolic disease as a disruption of the body's energy-regulation circuits. Hormones like insulin decide how fuel is handled... and underlying inflammation pushes people toward insulin resistance and vascular disease. You can't fix one pillar and ignore the rest."

Clinicians emphasise that long-term weight stability requires more than just appetite suppression. Structured nutrition, protein targets, resistance training, consistent sleep, electrolyte and renal monitoring matter as much. Doctors are also grappling with another question: what happens when the drug is stopped? Few are counselled on exit or maintenance strategies, raising the risk of rebound weight gain and the consequent emotional distress.

Psychiatric care is thus becoming central to metabolic medicine. Dr Sameer Malhotra, head of mental health and behavioural sciences at Max Super Speciality Hospital, Saket, New Delhi, cautions that rapid body change can trigger emotional destabilisation, fear of weight regain or psychological dependence on medication.

"CBT-E (cognitive behaviour therapy for eating disorders) helps patients normalise eating, manage emotional triggers and address the fear of weight gain," says Dr Pallavi Sharma, psychiatrist at Fortis Escorts, New Delhi, stressing the importance of psychological assessment before starting metabolic drugs.

Nutrition remains foundational. "Patients eat too little protein and too many refined carbohydrates," says Kalpana Gupta, clinical nutritionist at Max Saket. Because GLP-1 drugs suppress appetite powerfully, supervised nutrition is essential to prevent muscle loss, fatigue and micronutrient deficiencies. Dr Biware stresses that this is especially relevant in India. "Our diets are dominated by refined grains and sugars, which raise glycaemic load and make metabolic reversal harder. Simple, culturally familiar shifts—millets, pulses, legumes, balanced plates with protein and healthy fats—can blunt glucose spikes."

"Not every overweight person needs a GLP-1 drug," cautions Dr Misra. "Clinical thresholds and cardiometabolic risk must guide treatment." Taken together, the lesson is clear: metabolic medicines work—but only when used as part of a whole-body, long-term metabolic strategy, not as a standalone fix or a shortcut around food. ■